Clinical Practice Guideline: Acute Lateral Ankle Sprain Rehabilit

PulseAI Clinical Reference Library — Evidence-Based Patient Information

1. CLINICAL OVERVIEW & MECHANISM

Lateral ligament sprains typically involve inversion stress damaging the anterior talofibular ligament (ATFL) and calcaneofibular ligament (CFL). Clinical assessment rules out malleolar fracture using the Ottawa Ankle Rules.

2. PHASE 1 ACUTE MANAGEMENT (FIRST 48-72 HOURS) — P.O.L.I.C.E.

Protection: Semi-rigid ankle brace or supportive taping. Optimal Loading: Gentle active ankle circles from Day 1 to promote collagen alignment. Ice: Apply cold packs wrapped in a towel for 15-20 minutes every 2-3 hours. Compression: Elastic tubular bandage to limit edema. Elevation: Rest foot above heart level.

3. ANALGESIC & ANTI-INFLAMMATORY MANAGEMENT

Short-term oral NSAIDs (Ibuprofen 400mg TID with food) combined with Paracetamol 1000mg QDS provide effective analgesia during the first 3 to 5 days. Avoid rigid plaster cast immobilization; early functional weight-bearing accelerates recovery.

4. RED FLAG SIGNS REQUIRING URGENT CLINICAL REVIEW

Seek urgent medical re-evaluation if there is persistent inability to bear any weight after 72 hours, numbness or tingling in the toes, a cold pale foot, or severe calf tenderness suggestive of deep vein thrombosis (DVT).

Document issued for clinical guidance and patient education. Consult your attending physician for personalized care.